BASELINE → TREATMENT → FOLLOW-UP

Checks through your treatment

Baseline preparation, checks before each cycle and tests arranged because of symptoms are different. No fixed scan interval is imposed here.

Check	Before starting	Each 21-day cycle	Between visits / six-week checks	Benefit and liver review	After final treatment
Blood count, kidney, liver and glucose	Baseline tests	Repeat before every cycle	Earlier tests for symptoms or abnormalities	Distinguish underlying disease from treatment effects	Follow unresolved abnormalities
Varices, pressure and urine	Varices screening/management; blood pressure (BP) and dipstick	Dipstick before each cycle; BP before and after treatment	24-hour urine if indicated; report bleeding/headache	Review bleeding, BP, kidney and procedure risks	Monitor until risk/abnormalities resolve
Thyroid / immune symptoms	Baseline thyroid tests and symptom review	Review symptoms and steroid/ replacement medicines	thyroid function tests (TFTs) every six weeks; extra endocrine tests if indicated	Confirm recovery and any restart criteria	Immune effects may start or continue after stopping
Cardiac / anticoagulation	Cardiac history; consider echocardiogram (heart ultrasound) (ECHO) after chest radiation/ anthracycline	international normalised ratio (blood-clotting test) (INR) if indicated; warfarin plan	Weekly INR until warfarin dose stable, then each cycle	Investigate new cardiac symptoms urgently	Follow lasting effects

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00831 v2a · official regimen

Tests in your plan

Alpha-fetoprotein (AFP)

If part of your plan

WHEN

If it was raised and is informative for your tumour; your team sets the dates.

A selected marker interpreted with imaging and the clinical review.

Other liver conditions can raise AFP, and a normal result does not exclude cancer.

THE WHOLE TREATMENT JOURNEY

Atezolizumab + bevacizumab for hepatocellular carcinoma (HCC): treatment and checks

First-line advanced or unresectable liver cancer (HCC)

Atezolizumab is given first on Day 1 as an intravenous (IV) infusion or a subcutaneous injection into the thigh, followed by bevacizumab through a vein. Treatment is planned every 21 days while it is helping and side effects remain acceptable.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Each 21-day cycle	Between visits / six-week checks	Benefit and liver review	After final treatment
Treatment course NCCP 00831	Confirm first-line HCC and route	Atezolizumab IV/subcutaneous (SC) first, then IV bevacizumab	Review toxicities; no take-home anticancer tablets	Assess each component separately	Agree follow-up / next treatment
Blood count, kidney, liver and glucose NCCP 00831	Baseline tests	Repeat before every cycle	Earlier tests for symptoms or abnormalities	Distinguish underlying disease from treatment effects	Follow unresolved abnormalities
Varices, pressure and urine NCCP 00831	Varices screening/management; BP and dipstick	Dipstick before each cycle; BP before and after treatment	24-hour urine if indicated; report bleeding/headache	Review bleeding, BP, kidney and procedure risks	Monitor until risk/abnormalities resolve
Thyroid / immune symptoms NCCP 00831	Baseline thyroid tests and symptom review	Review symptoms and steroid/ replacement medicines	TFTs every six weeks; extra endocrine tests if indicated	Confirm recovery and any restart criteria	Immune effects may start or continue after stopping
Cardiac / anticoagulation NCCP 00831	Cardiac history; consider ECHO after chest radiation/anthracycline	INR if indicated; warfarin plan	Weekly INR until warfarin dose stable, then each cycle	Investigate new cardiac symptoms urgently	Follow lasting effects

What each check is for

1 Varices review before bevacizumab

Before first dose

Liver disease and anti-VEGF therapy can combine to increase bleeding risk.

Ask / **act:** Confirm the screening result and management plan.

Sources: NCCP 00831

2 Review each medicine

Before every cycle

Immune and anti-VEGF toxicities have different actions.

Ask / **act:** Record which medicine is held, continued or stopped.

Sources: NCCP 00831

3 Six-week thyroid monitoring

During treatment

Thyroid dysfunction can develop without obvious early symptoms.

Ask / **act:** Confirm who books and reviews results.

Sources: NCCP 00831

4 After treatment

After last dose

Immune effects may be delayed and other risks persist.

Ask / **act:** Agree contact, contraception and medicine follow-up.

Sources: NCCP 00831

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-29 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.24